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April 23, 2026

ENGROSSED HOUSE
BILL NO. 4202

By: Schreiber, Deck, and
Dollens of the House

and

Gollihare of the Senate

An Act relating to workers' compensation; amending 85A O.S. 2021, Section 50, which relates to fee schedules; modifying reimbursement rate provision regarding magnetic resonance imaging; and declaring an emergency.

BE IT ENACTED BY THE PEOPLE OF THE STATE OF OKLAHOMA:

SECTION 1. AMENDATORY 85A O.S. 2021, Section 50, is amended to read as follows:

Section 50. A. The employer shall promptly provide an injured employee with medical, surgical, hospital, optometric, podiatric, chiropractic and nursing services, along with any medicine, crutches, ambulatory devices, artificial limbs, eyeglasses, contact lenses, hearing aids, and other apparatus as may be reasonably necessary in connection with the injury received by the employee. The employer shall have the right to choose the treating physician or chiropractor.

B. If the employer fails or neglects to provide medical treatment within five (5) days after actual knowledge is received of

1 an injury, the injured employee may select a physician or
2 chiropractor to provide medical treatment at the expense of the
3 employer; provided, however, that the injured employee, or another
4 in the employee's behalf, may obtain emergency treatment at the
5 expense of the employer where such emergency treatment is not
6 provided by the employer.

7 C. Diagnostic tests shall not be repeated sooner than six (6)
8 months from the date of the test unless agreed to by the parties or
9 ordered by the Commission for good cause shown.

10 D. Unless recommended by the treating doctor or chiropractor at
11 the time claimant reaches maximum medical improvement or by an
12 independent medical examiner, continuing medical maintenance shall
13 not be awarded by the Commission. The employer or insurance carrier
14 shall not be responsible for continuing medical maintenance or pain
15 management treatment that is outside the parameters established by
16 the Physician Advisory Committee or ODG. The employer or insurance
17 carrier shall not be responsible for continuing medical maintenance
18 or pain management treatment not previously ordered by the
19 Commission or approved in advance by the employer or insurance
20 carrier.

21 E. An employee claiming or entitled to benefits under the
22 Administrative Workers' Compensation Act, shall, if ordered by the
23 Commission or requested by the employer or insurance carrier, submit
24 himself or herself for medical examination. If an employee refuses

1 to submit himself or herself to examination, his or her right to
2 prosecute any proceeding under the Administrative Workers'
3 Compensation Act shall be suspended, and no compensation shall be
4 payable for the period of such refusal.

5 F. For compensable injuries resulting in the use of a medical
6 device, ongoing service for the medical device shall be provided in
7 situations including, but not limited to, medical device battery
8 replacement, ongoing medication refills related to the medical
9 device, medical device repair, or medical device replacement.

10 G. The employer shall reimburse the employee for the actual
11 mileage in excess of twenty (20) miles round trip to and from the
12 employee's home to the location of a medical service provider for
13 all reasonable and necessary treatment, for an evaluation of an
14 independent medical examiner and for any evaluation made at the
15 request of the employer or insurance carrier. The rate of
16 reimbursement for such travel expense shall be the official
17 reimbursement rate as established by the State Travel Reimbursement
18 Act. In no event shall the reimbursement of travel for medical
19 treatment or evaluation exceed six hundred (600) miles round trip.

20 H. Fee Schedule.

21 1. The Commission shall conduct a review and update of the
22 Current Procedural Terminology (CPT) in the Fee Schedule every two
23 (2) years pursuant to the provisions of paragraph 14 of this
24 subsection. The Fee Schedule shall establish the maximum rates that

1 medical providers shall be reimbursed for medical care provided to
2 injured employees including, but not limited to, charges by
3 physicians, chiropractors, dentists, counselors, hospitals,
4 ambulatory and outpatient facilities, clinical laboratory services,
5 diagnostic testing services, and ambulance services, and charges for
6 durable medical equipment, prosthetics, orthotics, and supplies.
7 The most current Fee Schedule established by the Administrator of
8 the Workers' Compensation Court prior to February 1, 2014, shall
9 remain in effect, unless or until the Legislature approves the
10 Commission's proposed Fee Schedule.

11 2. Reimbursement for medical care shall be prescribed and
12 limited by the Fee Schedule. The director of the Employees Group
13 Insurance Division of the Office of Management and Enterprise
14 Services shall provide the Commission such information as may be
15 relevant for the development of the Fee Schedule. The Commission
16 shall develop the Fee Schedule in a manner in which quality of
17 medical care is assured and maintained for injured employees. The
18 Commission shall give due consideration to additional requirements
19 for physicians treating an injured worker under the Administrative
20 Workers' Compensation Act, including, but not limited to,
21 communication with claims representatives, case managers, attorneys,
22 and representatives of employers, and the additional time required
23 to complete forms for the Commission, insurance carriers, and
24 employers.

1 3. In making adjustments to the Fee Schedule, the Commission
2 shall use, as a benchmark, the reimbursement rate for each Current
3 Procedural Terminology (CPT) code provided for in the fee schedule
4 published by the Centers for Medicare and Medicaid Services of the
5 U.S. Department of Health and Human Services for use in Oklahoma
6 (Medicare Fee Schedule) on the effective date of this section,
7 workers' compensation fee schedules employed by neighboring states,
8 the latest edition of "Relative Values for Physicians" (RVP), usual,
9 customary and reasonable medical payments to workers' compensation
10 health care providers in the same trade area for comparable
11 treatment of a person with similar injuries, and all other data the
12 Commission deems relevant. For services not valued by CMS, the
13 Commission shall establish values based on the usual, customary and
14 reasonable medical payments to health care providers in the same
15 trade area for comparable treatment of a person with similar
16 injuries.

- 17 a. No reimbursement shall be allowed for any magnetic
18 resonance imaging (MRI) unless the MRI is provided by
19 an entity that meets Medicare requirements for the
20 payment of MRI services or is accredited by the
21 American College of Radiology, the Intersocietal
22 Accreditation Commission or the Joint Commission on
23 Accreditation of Healthcare Organizations. ~~For all~~
24 ~~other radiology procedures, the reimbursement rate~~

1 ~~shall be the lesser of the reimbursement rate allowed~~
2 ~~by the 2010 Oklahoma Fee Schedule and two hundred~~
3 ~~seven percent (207%) of the Medicare Fee Schedule.~~

4 b. For reimbursement of medical services for Evaluation
5 and Management of injured employees as defined in the
6 Fee Schedule adopted by the Commission, the
7 reimbursement rate shall not be less than one hundred
8 fifty percent (150%) of the Medicare Fee Schedule.

9 c. Any entity providing durable medical equipment,
10 prosthetics, orthotics or supplies shall be accredited
11 by a CMS-approved accreditation organization. If a
12 physician provides durable medical equipment,
13 prosthetics, orthotics, prescription drugs, or
14 supplies to a patient ancillary to the patient's
15 visit, reimbursement shall be no more than ten percent
16 (10%) above cost.

17 d. The Commission shall develop a reasonable stop-loss
18 provision of the Fee Schedule to provide for adequate
19 reimbursement for treatment for major burns, severe
20 head and neurological injuries, multiple system
21 injuries, and other catastrophic injuries requiring
22 extended periods of intensive care. An employer or
23 insurance carrier shall have the right to audit the
24 charges and question the reasonableness and necessity

1 of medical treatment contained in a bill for treatment
2 covered by the stop-loss provision.

3 4. The right to recover charges for every type of medical care
4 for injuries arising out of and in the course of covered employment
5 as defined in the Administrative Workers' Compensation Act shall lie
6 solely with the Commission. When a medical care provider has
7 brought a claim to the Commission to obtain payment for services, a
8 party who prevails in full on the claim shall be entitled to
9 reasonable attorney fees.

10 5. Nothing in this section shall prevent an employer, insurance
11 carrier, group self-insurance association, or certified workplace
12 medical plan from contracting with a provider of medical care for a
13 reimbursement rate that is greater than or less than limits
14 established by the Fee Schedule.

15 6. A treating physician may not charge more than Four Hundred
16 Dollars (\$400.00) per hour for preparation for or testimony at a
17 deposition or appearance before the Commission in connection with a
18 claim covered by the Administrative Workers' Compensation Act.

19 7. The Commission's review of medical and treatment charges
20 pursuant to this section shall be conducted pursuant to the Fee
21 Schedule in existence at the time the medical care or treatment was
22 provided. The judgment approving the medical and treatment charges
23 pursuant to this section shall be enforceable by the Commission in
24

1 the same manner as provided in the Administrative Workers'
2 Compensation Act for the enforcement of other compensation payments.

3 8. Charges for prescription drugs dispensed by a pharmacy shall
4 be limited to ninety percent (90%) of the average wholesale price of
5 the prescription, plus a dispensing fee of Five Dollars (\$5.00) per
6 prescription. "Average wholesale price" means the amount determined
7 from the latest publication designated by the Commission.

8 Physicians shall prescribe and pharmacies shall dispense generic
9 equivalent drugs when available. If the National Drug Code, or
10 "NDC", for the drug product dispensed is for a repackaged drug, then
11 the maximum reimbursement shall be the lesser of the original
12 labeler's NDC and the lowest-cost therapeutic equivalent drug
13 product. Compounded medications shall be billed by the compounding
14 pharmacy at the ingredient level, with each ingredient identified
15 using the applicable NDC of the drug product, and the corresponding
16 quantity. Ingredients with no NDC area are not separately
17 reimbursable. Payment shall be based on a sum of the allowable fee
18 for each ingredient plus a dispensing fee of Five Dollars (\$5.00)
19 per prescription.

20 9. When medical care includes prescription drugs dispensed by a
21 physician or other medical care provider and the NDC for the drug
22 product dispensed is for a repackaged drug, then the maximum
23 reimbursement shall be the lesser of the original labeler's NDC and
24 the lowest-cost therapeutic equivalent drug product. Payment shall

1 be based upon a sum of the allowable fee for each ingredient plus a
2 dispensing fee of Five Dollars (\$5.00) per prescription. Compounded
3 medications shall be billed by the compounding pharmacy.

4 10. Implantables are paid in addition to procedural
5 reimbursement paid for medical or surgical services. A
6 manufacturer's invoice for the actual cost to a physician, hospital
7 or other entity of an implantable device shall be adjusted by the
8 physician, hospital or other entity to reflect, at the time
9 implanted, all applicable discounts, rebates, considerations and
10 product replacement programs and shall be provided to the payer by
11 the physician or hospital as a condition of payment for the
12 implantable device. If the physician, or an entity in which the
13 physician has a financial interest other than an ownership interest
14 of less than five percent (5%) in a publically traded company,
15 provides implantable devices, this relationship shall be disclosed
16 to patient, employer, insurance company, third-party commission,
17 certified workplace medical plan, case managers, and attorneys
18 representing claimant and defendant. If the physician, or an entity
19 in which the physician has a financial interest other than an
20 ownership interest of less than five percent (5%) in a publicly
21 traded company, buys and resells implantable devices to a hospital
22 or another physician, the markup shall be limited to ten percent
23 (10%) above cost.

1 11. Payment for medical care as required by the Administrative
2 Workers' Compensation Act shall be due within forty-five (45) days
3 of the receipt by the employer or insurance carrier of a complete
4 and accurate invoice, unless the employer or insurance carrier has a
5 good-faith reason to request additional information about such
6 invoice. Thereafter, the Commission may assess a penalty up to
7 twenty-five percent (25%) for any amount due under the Fee Schedule
8 that remains unpaid on the finding by the Commission that no good-
9 faith reason existed for the delay in payment. If the Commission
10 finds a pattern of an employer or insurance carrier willfully and
11 knowingly delaying payments for medical care, the Commission may
12 assess a civil penalty of not more than Five Thousand Dollars
13 (\$5,000.00) per occurrence.

14 12. If an employee fails to appear for a scheduled appointment
15 with a physician or chiropractor, the employer or insurance company
16 shall pay to the physician or chiropractor a reasonable charge, to
17 be determined by the Commission, for the missed appointment. In the
18 absence of a good-faith reason for missing the appointment, the
19 Commission shall order the employee to reimburse the employer or
20 insurance company for the charge.

21 13. Physicians or chiropractors providing treatment under the
22 Administrative Workers' Compensation Act shall disclose under
23 penalty of perjury to the Commission, on a form prescribed by the
24 Commission, any ownership or interest in any health care facility,

1 business, or diagnostic center that is not the physician's or
2 chiropractor's primary place of business. The disclosure shall
3 include any employee leasing arrangement between the physician or
4 chiropractor and any health care facility that is not the
5 physician's or chiropractor's primary place of business. A
6 physician's or chiropractor's failure to disclose as required by
7 this section shall be grounds for the Commission to disqualify the
8 physician or chiropractor from providing treatment under the
9 Administrative Workers' Compensation Act.

10 14. a. Beginning on May 28, 2019, the Commission shall
11 conduct an evaluation of the Fee Schedule, which shall
12 include an update of the list of Current Procedural
13 Terminology (CPT) codes, a line item adjustment or
14 renewal of all rates, and amendment as needed to the
15 rules applicable to the Fee Schedule.

16 b. The Commission shall contract with an external
17 consultant with knowledge of workers' compensation fee
18 schedules to review regional and nationwide
19 comparisons of Oklahoma's Fee Schedule rates and date
20 and market for medical services. The consultant shall
21 receive written and oral comment from employers,
22 workers' compensation medical service and insurance
23 providers, self-insureds, group self-insurance
24 associations of this state and the public. The

1 consultant shall submit a report of its findings and a
2 proposed amended Fee Schedule to the Commission.

3 c. The Commission shall adopt the proposed amended Fee
4 Schedule in whole or in part and make any additional
5 updates or adjustments. The Commission shall submit a
6 proposed updated and adjusted Fee Schedule to the
7 President Pro Tempore of the Senate, the Speaker of
8 the House of Representatives and the Governor. The
9 proposed Fee Schedule shall become effective on July 1
10 following the legislative session, if approved by
11 Joint Resolution of the Legislature during the session
12 in which a proposed Fee Schedule is submitted.

13 d. Beginning on May 28, 2019, an external evaluation
14 shall be conducted and a proposed amended Fee Schedule
15 shall be submitted to the Legislature for approval
16 during the 2020 legislative session. Thereafter, an
17 external evaluation shall be conducted and a proposed
18 amended Fee Schedule shall be submitted to the
19 Legislature for approval every two (2) years.

20 I. Formulary. The Commission by rule shall adopt a closed
21 formulary. Rules adopted by the Commission shall allow an appeals
22 process for claims in which a treating doctor determines and
23 documents that a drug not included in the formulary is necessary to
24 treat an injured employee's compensable injury. The Commission by

1 rule shall require the use of generic pharmaceutical medications and
2 clinically appropriate over-the-counter alternatives to prescription
3 medications unless otherwise specified by the prescribing doctor, in
4 accordance with applicable state law.

5 SECTION 2. It being immediately necessary for the preservation
6 of the public peace, health or safety, an emergency is hereby
7 declared to exist, by reason whereof this act shall take effect and
8 be in full force from and after its passage and approval.

9 COMMITTEE REPORT BY: COMMITTEE ON BUSINESS AND INSURANCE
10 April 23, 2026 - DO PASS
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